

REGULATORY DOSSIER SUBMISSION

Dossier ID: DOS-79616291C0

Country: Tanzania

Submission Date: 2026-01-24

Product: Nifemed (nifedipine) | ATC: C08CA05 | Strength: 30 mg

Applicant: Global Therapeutics | Manufacturer: Entebbe Pharma Solutions

Policy Label: reject_and_return | Risk Score: 0.92

AMR Stewardship: AWaRe=not_applicable | Unmet Need=not_applicable | GLASS Trend=not_applicable
| Watch Similarity=not_applicable

SECTION CONTENT

[1] m1_application_admin - Application Form and Administrative Information

Labels: presence=present; length=length_ok; correctness=correct

This dossier constitutes a formal application for Marketing Authorization of Nifemed (nifedipine) submitted on 2026-01-24 to the national regulatory authority of Tanzania. The applicant, Global Therapeutics, formally requests approval for the commercial distribution of this medicinal product. The proposed pharmaceutical form is tablet with a strength of 30 mg, classified under ATC code C08CA05. All required administrative declarations, legal attestations, letters of authorization, and regional application forms have been completed, signed by the responsible Qualified Person, and appended to Module 1. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The applicant confirms alignment with applicable technical guidance and regional submission standards. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The proposed prescribing information and patient information leaflet have been translated into local languages. A comprehensive table of contents and glossary of abbreviations are provided to facilitate regulatory review. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Method validation records and quality controls were reviewed for internal consistency. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demons

[1] m1_manufacturer_gmp - Manufacturer and GMP Evidence

Labels: presence=present; length=length_ok; correctness=partial

Error tags: gmp_outdated

Latest GMP inspection date is 2022-01-02. No recent inspection evidence was provided for the required review window. Method validation records and quality controls were reviewed for internal consistency. Batch manufacturing records and standard operating procedures (SOPs) are maintained and archived on-site. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Site quality-system commitments were compared against the proposed commercial manufacturing activities. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. All referenced documents are available in the dossier annex and traceable by section identifier. The applicant confirms alignment with applicable technical guidance and regional submission standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities

(MedDRA) version 24.0. Recent regulatory inspections yielded no critical observations, and all major findings have been effectively closed. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Inspection history, CAPA closure records, and site responsibilities were reconciled across the manufacturi

[1] m1_product_information - Product Information and Naming

Labels: presence=present; length=length_ok; correctness=correct

The proposed proprietary name for this medicinal product is Nifemed, containing the active pharmaceutical ingredient nifedipine. The draft Summary of Product Characteristics (SmPC), Patient Information Leaflet (PIL), and primary/secondary packaging labels are provided for review. These documents detail the approved therapeutic indications, posology, contraindications, special warnings, and precautions for use. A comprehensive risk management plan to minimize medication errors is also submitted. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcar

[2] m2_quality_overall_summary - Quality Overall Summary

Labels: presence=present; length=length_ok; correctness=correct

This Quality Overall Summary (QOS) provides a critical evaluation of the chemistry, manufacturing, and controls (CMC) data for nifedipine. The control strategy justifies the proposed specifications for both the active substance and the finished product, emphasizing critical quality attributes (CQAs) and critical process parameters (CPPs). Impurity profiles, including degradation products and potential genotoxic impurities, have been thoroughly characterized. Batch analysis data from three commercial-scale validation batches demonstrate consistent manufacturing performance and compliance with all release criteria. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. All referenced documents are available in the dossier annex and traceable by section identifier. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Method validation records and quality controls were reviewed for internal consistency. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The applicant confirms alignment with applicable technical guidance and regional submission standards. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The applicant confirms alignment with applicable technical guidance and regional submission standards. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. All referenced documents are available in the dossier annex and traceable by section identifier. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. A randomized, do

[2] m2_clinical_overview - Clinical Overview and Benefit-Risk Summary

Labels: presence=present; length=length_ok; correctness=incorrect

Error tags: clinical_missing

Clinical overview for type 2 diabetes indicates pivotal efficacy data are not available at submission time. Benefit-risk cannot be concluded pending full clinical study reports and verified endpoint analyses. No new safety signals were identified during the extended open-label follow-up period. All referenced documents are available in the dossier annex and traceable by section identifier. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Method validation records and quality controls were reviewed for internal consistency. The clinical overview links the therapeutic rationale to the pivotal evidence package and the proposed indication. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Clinical interpretation focuses on whether efficacy, safety, and benefit-risk conclusions are supported by the submitted evidence. The applicant confirms alignment with applicable technical guidance and regional submission standards. Benefit-risk language was reviewed for consistency with the submitted efficacy and safety summaries. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating ac

[3] m3_api_quality - API Quality and Control Strategy

Labels: presence=present; length=length_ok; correctness=correct

The active pharmaceutical ingredient (API) section details the complete synthesis route, starting from well-defined regulatory starting materials. Robust in-process controls and intermediate specifications ensure the consistent quality of the final drug substance. The analytical procedures used for release and stability testing have been fully validated for accuracy, precision, linearity, and robustness. The impurity control strategy adequately addresses organic impurities, residual solvents, and elemental impurities in accordance with current ICH guidelines. Forced degradation studies confirm the stability-indicating nature of the assay methods. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Batch analysis, validation, and impurity management records support the proposed API quality framework. All referenced documents are available in the dossier annex and traceable by section identifier. Data were analyzed using a mixed-effects model for

[3] m3_fpp_manufacturing - FPP Manufacturing Process and Controls

Labels: presence=present; length=length_ok; correctness=correct

The finished pharmaceutical product (FPP) manufacturing process utilizes standard, scalable unit operations. A formal quality risk assessment (e.g., FMEA) was conducted to identify critical process parameters (CPPs) that impact critical quality attributes (CQAs). Process performance qualification (PPQ) reports for three consecutive commercial-scale batches verify that the process is maintained in a state of control. In-process controls, intermediate hold-

time studies, and packaging validation data are presented to substantiate the robustness and reproducibility of the commercial manufacturing operations. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The applicant confirms alignment with applicable technical guidance and regional submission standards. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Method validation records and quality controls were reviewed for internal consistency. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. All referenced documents are available in the dossier annex and traceable by section identifier. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The applicant confirms alignment with applicable technical guidance and regional submission standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The container

[3] m3_stability - Stability and Shelf-Life Justification

Labels: presence=present; length=length_ok; correctness=correct

A comprehensive stability program has been executed to justify the proposed shelf-life and storage conditions. Data from both accelerated (40°C/75% RH for 6 months) and long-term storage conditions (spanning up to 36 months) are presented for multiple primary stability batches. Statistical evaluation using linear regression and poolability tests confirms that degradation trends remain well within the acceptable specification limits. Photostability and freeze-thaw cycling studies demonstrate that the product does not require specific handling precautions. A post-approval stability protocol commitment is included. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The applicant confirms alignment with applicable technical guidance and regional submission standards. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. All referenced documents are available in the dossier annex and traceable by section identifier. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Pharmacokinetic parameters were derived using non-

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[4] m4_nonclinical_summary - Nonclinical Study Summary

Labels: presence=present; length=length_ok; correctness=correct

The nonclinical testing program evaluated the primary pharmacodynamics, secondary pharmacodynamics, safety pharmacology, and comprehensive toxicology profile of the compound. Repeat-dose toxicity studies in two mammalian species established clear no-observed-adverse-effect levels (NOAELs). Genotoxicity testing (Ames and in vivo micronucleus assays) yielded negative results. Reproductive and developmental toxicity studies revealed no teratogenic signals at clinically relevant exposures. The calculated safety margins support the proposed clinical dosing regimen without major safety concerns. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The applicant confirms alignment with applicable technical guidance and regional submission standards. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. All referenced documents are available in the dossier annex and traceable by section identifier. Method validation records and quality controls were reviewed for internal consistency. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Statistical significance was set at an alpha level of 0.05.

[5] m5_trial_listing - Tabular Listing of Clinical Studies

Labels: presence=present; length=length_ok; correctness=correct

Tabular listing indicates planned or ongoing studies, but no completed pivotal study reports were provided in this submission package. All referenced documents are available in the dossier annex and traceable by section identifier. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The applicant confirms alignment with applicable technical guidance and regional submission standards. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Method validation records and quality controls were reviewed for internal consistency. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Statistical significance was set at an alpha level of 0.05.

[5] m5_pivotal_trial_reports - Pivotal Clinical Trial Reports

Labels: presence=missing; length=missing; correctness=incorrect

Error tags: clinical_missing

[MISSING SECTION]

[5] m5_bioequivalence - Biopharmaceutics and Bioequivalence Evidence

Labels: presence=present; length=too_short; correctness=partial

Error tags: insufficient_detail

Insufficient detail provided in this section.